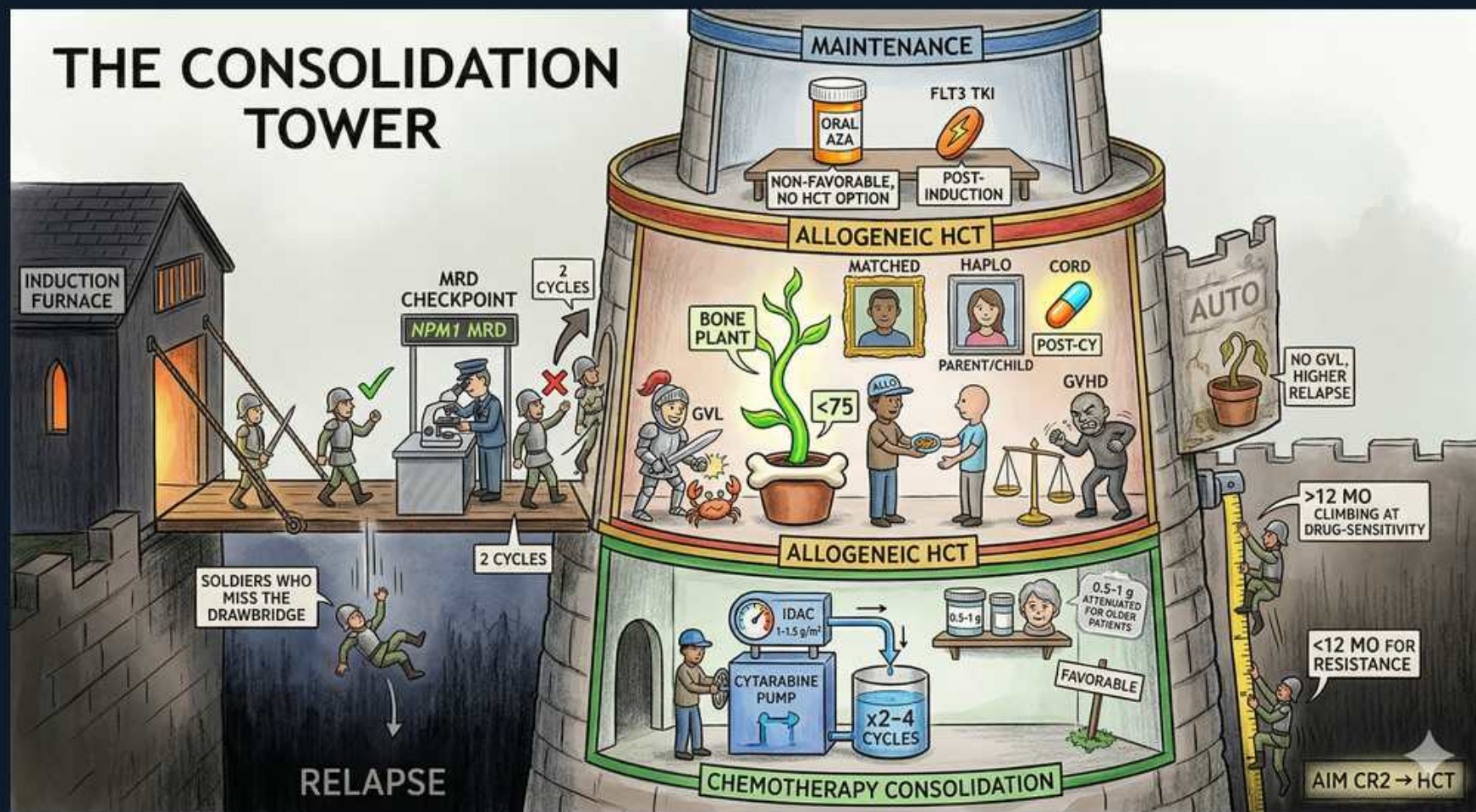


THE CONSOLIDATION TOWER



1. Consolidation Tower

WHAT YOU SEE

'THE CONSOLIDATION TOWER' title over a fortress.

WHAT IT ENCODES

After induction achieves complete remission, consolidation eradicates residual disease. The choice is chemotherapy consolidation (favorable risk) vs allogeneic HCT (intermediate/adverse, fit patients).

BOARD TRAP

Consolidation is mandatory after CR — achieving remission alone is not curative. Stopping at induction relapses nearly everyone.

2. Induction furnace

WHAT YOU SEE

'INDUCTION FURNACE' building on the left.

WHAT IT ENCODES

Induction (7+3) precedes consolidation. The bone-marrow blast clearance and count recovery define complete remission before consolidation begins.

BOARD TRAP

Don't consolidate a patient who never achieved CR; a non-responder needs reinduction/salvage, not consolidation.

3. MRD checkpoint

WHAT YOU SEE

'MRD CHECKPOINT' with 'NPM1 / MRD' and a drawbridge.

WHAT IT ENCODES

MRD assessment (NPM1 PCR, flow) after induction/consolidation predicts relapse and modifies the transplant decision. MRD-positive favorable patients may be escalated toward transplant.

BOARD TRAP

MRD positivity changes management even in 'favorable' disease — it is not just a research metric.

4. Soldiers miss drawbridge

WHAT YOU SEE

'SOLDIERS WHO MISS THE DRAWBRIDGE' falling toward 'RELAPSE'.

WHAT IT ENCODES

Failure to clear MRD or inadequate consolidation leads to relapse. Relapsed AML has poor prognosis and pushes toward salvage chemo then allo-HCT.

BOARD TRAP

Relapsed/refractory AML in a fit patient aims for re-remission then transplant; chemo alone rarely cures relapse.

5. Chemo consolidation

WHAT YOU SEE

Green base 'CHEMOTHERAPY CONSOLIDATION'.

WHAT IT ENCODES

For favorable-risk AML: intermediate/high-dose cytarabine for multiple cycles. Avoids transplant toxicity while delivering durable remission in chemo-sensitive disease.

BOARD TRAP

High-dose cytarabine toxicity: cerebellar dysfunction (check nystagmus/ataxia before each dose) and conjunctivitis (steroid eye drops) — dose-reduce in renal impairment and the elderly.

6. Cytarabine pump x2-4

WHAT YOU SEE

'CYTARABINE PUMP' and 'x2-4 CYCLES'.

WHAT IT ENCODES

Roughly 2–4 cycles of cytarabine-based consolidation for favorable disease. HiDAC particularly benefits CBF AML.

BOARD TRAP

Cerebellar toxicity is dose- and age-dependent; hold the dose for new ataxia/nystagmus rather than pushing through.

7. Allogeneic HCT band

WHAT YOU SEE

Orange 'ALLOGENEIC HCT' band wrapping the tower.

WHAT IT ENCODES

Allo-HCT is standard consolidation for intermediate (selected) and adverse-risk fit patients in CR1. The graft-versus-leukemia effect provides the durable anti-leukemic benefit.

BOARD TRAP

It is graft-versus-LEUKEMIA, not just the conditioning chemo, that cures — which is why autologous transplant is inferior here.

8. Non-favorable → HCT

WHAT YOU SEE

'NON-FAVORABLE — NO HCT OPTION' / post-induction box.

WHAT IT ENCODES

Non-favorable risk groups proceed to allo-HCT once in remission if fit and a donor exists. Bridging therapy maintains CR until transplant.

BOARD TRAP

Adverse risk without transplant is undertreatment in a fit patient — donor search should start early, at diagnosis for adverse disease.

9. Donor sources

WHAT YOU SEE

'MATCHED / HAPLO / CORD / POST-CY' tiles with faces.

WHAT IT ENCODES

Donor options: HLA-matched sibling or unrelated, haploidentical (with post-transplant cyclophosphamide), or cord blood. Post-Cy reduces GVHD in haplo transplants.

BOARD TRAP

Lack of a matched sibling is no longer a barrier — haplo with post-transplant cyclophosphamide expands eligibility to nearly all patients.

10. GVL vs GVHD scale

WHAT YOU SEE

'GVL' sword vs 'GVHD' on a balance, '<75'.

WHAT IT ENCODES

Graft-versus-leukemia is therapeutic; graft-versus-host disease is the toxicity. Reduced-intensity conditioning extends transplant to older/comorbid patients (roughly <75 / fitness-based).

BOARD TRAP

Reduced-intensity conditioning lowers toxicity but raises relapse risk because it relies more on GVL — note the 'NO GVL → HIGHER RELAPSE' sign.

11. Maintenance — oral AZA

WHAT YOU SEE

'MAINTENANCE' cap with 'ORAL AZA'.

WHAT IT ENCODES

Oral azacitidine (CC-486) maintenance prolongs survival in AML patients in CR1 after intensive chemo who are not going to transplant. Distinct from injectable azacitidine.

BOARD TRAP

Oral azacitidine (CC-486) is NOT bioequivalent to subcutaneous azacitidine — they are different formulations with different indications.

12. FLT3 TKI maintenance

WHAT YOU SEE

'FLT3 TKI' and 'POST-INDUCTION' tags.

WHAT IT ENCODES

FLT3-mutated AML: a FLT3 inhibitor is added to induction (midostaurin) and may continue as maintenance, including post-transplant maintenance to reduce relapse.

BOARD TRAP

Midostaurin frontline vs gilteritinib/sorafenib in maintenance/relapse — match the FLT3 drug to the phase of therapy.

13. CR2 → HCT

WHAT YOU SEE

'AIM CR2 → HCT' at bottom right.

WHAT IT ENCODES

At relapse, the goal is to achieve a second remission (CR2) and proceed to allo-HCT, which offers the only realistic chance of cure in relapsed disease.

BOARD TRAP

Relapsed AML is not consolidated with chemo alone; the target is CR2 followed by transplant in eligible patients.